

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

January 15, 2020

MEMORANDUM FOR:

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Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C)

THROUGH:

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Chief of Staff
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CAPT (b)(6),(b)(7)(C) RN
Chief, Investigations Unit
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FROM:

CDR (b)(6),(b)(7)(C) RN
Investigator
ICE Health Service Corps

(b)(6),(b)(7)(C)

SUBJECT:

Mortality Review – Report of Findings
Roberto RODRIGUEZ-Espinoza, (b)(6),(b)(7)(C)
(a.k.a. Candido RODRIGUEZ)

Executive Summary:

Mr. Roberto RODRIGUEZ-Espinoza, a 37-year-old Mexican male, was in U.S. Immigration and Customs Enforcement (ICE) custody from September 3 to 10, 2019. During his detention period, ICE housed Mr. RODRIGUEZ-Espinoza at McHenry County Adult Correctional Facility (MCACF) in Woodstock, Illinois (IL).

On September 3, 2019, an MCACF custody officer (CO) completed Mr. RODRIGUEZ-Espinoza's intake screening. Mr. RODRIGUEZ-Espinoza denied a history of medical or mental health conditions, but reported a history of consuming a six pack of alcohol daily. The CO did

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not inquire further about his alcohol usage, or observe any signs of alcohol withdrawal, and documented his appearance as “ok.” A licensed practical nurse (LPN) reviewed and initialed Mr. RODRIGUEZ-Espinoza’s intake screening form, but did not document any additional information regarding his alcohol usage, i.e., alcohol frequency, last day of consumption, and alcohol withdrawal history.

On September 7, 2019, at approximately 3:30 p.m., a CO observed Mr. RODRIGUEZ-Espinoza walking around his dorm confused and unfamiliar with his surroundings and notified the shift sergeant (Sgt.) and an LPN, who reported to the housing unit. During the LPN’s conversation with Mr. RODRIGUEZ-Espinoza, using interpreter services, he reported a history of consuming a six pack of beer and a quarter bottle of tequila, daily for 27 years. The LPN suspected alcohol withdrawal and completed a clinical institute withdrawal assessment for alcohol [(CIWA); a guide for evaluating the severity of alcohol withdrawal symptoms; scores range from absent to minimal (zero to nine) to severe (21 to 67)]. The LPN documented Mr. RODRIGUEZ-Espinoza’s CIWA score of 4, and his vital signs (VS) were: blood pressure (BP) - 130/84 (90/60 - 120/80), pulse (P) - 81 (60 to 100 beats per minute), temperature (T) - 97.8 (97.8 - 99.1), respirations (R) - 16 (12 to 20 breaths per minute), and oxygen saturation [(SpO₂); fraction of oxygen saturated hemoglobin in the blood] - 98 percent [(%) 95-100] while breathing room air.

The LPN notified the on-call medical doctor (MD), initiated MCACF’s alcohol detoxification protocol, administered chlordiazepoxide (a sedative used to treat alcohol abuse), and advised the officer to transfer Mr. RODRIGUEZ-Espinoza to the medical pod for ongoing medical observation. Later that evening, the shift Sgt. spoke to Mr. RODRIGUEZ-Espinoza, observed normal physical movement, clear speech, and persistent altered mental status.

- At approximately 9:45 p.m., the LPN contacted the on-call MD due to Mr. RODRIGUEZ-Espinoza’s altered mental status worsening. The LPN received a verbal order to transport him to the local hospital via custody transportation for a neurological evaluation and to rule out delirium tremens [(DTs); most severe form of alcohol withdrawal, which involves sudden and severe mental and/or nervous system changes].

On September 8, 2019, at approximately 10:45 p.m., MCAF transferred Mr. RODRIGUEZ-Espinoza to Northwestern Medicine Woodstock Hospital (NMWH) in Woodstock, IL, via custody transportation. Upon arrival to NMWH, an MD assessed Mr. RODRIGUEZ-Espinoza. He complained of a headache and nausea, and the MD ordered ondansetron (antiemetic medication), ketorolac (pain medication), and lorazepam (anti-anxiety medication). Mr. RODRIGUEZ-Espinoza’s VS were: BP 130/74, P 82, R 18, T 99, SpO₂ 100%, and a CIWA score of 25. The MD diagnosed Mr. RODRIGUEZ-Espinoza with acute alcoholic liver disease with fatty liver, alcohol withdrawal with DTs, irritability, agitation, and confusion, and transferred him to Northwestern Medicine Huntley Hospital (NMHH) in Huntley, IL, due to a severely altered mental status and for a higher level of care.

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- At approximately 3:10 a.m., Mr. RODRIGUEZ-Espinoza arrived at NMHH. An MD evaluated him, continued intravenous fluids with vitamin supplements, initiated CIWA protocols, and transferred him to the intensive care unit (ICU), due to severe alcohol withdrawal symptoms and heavy sedation, as a result of receiving multiple doses of lorazepam while at NMWH. His VS were BP 144/77, P 113, T 99.9 and SpO2 of 99.9%. Shortly after arriving at NMHH, Mr. RODRIGUEZ-Espinoza fell asleep and never regained consciousness.
- At approximately 5:30 p.m., the NMHH MD ordered a computed tomography [(CT); radiological imaging] scan, which revealed a subdural hematoma [(SDH); a collection of blood outside of the brain]. Mr. RODRIGUEZ-Espinoza required intubation and a neurosurgery evaluation. The NMHH MD arranged for Mr. RODRIGUEZ-Espinoza's transfer to Northwestern Medicine Central DuPage Hospital (NMCDH) in Winfield, IL, for a higher level of care (i.e., neurology evaluation).
- At approximately 9:00 p.m., Mr. RODRIGUEZ-Espinoza arrived at NMCDH. His VS were P 75, R 22, and a Glasgow Coma Scale rating of 3 [(GCS); neurological scale that objectively evaluates a person's consciousness; a minimum score of 3, indicates deep coma or brain dead, and a maximum score of 15, indicates fully awake]. A neurologist evaluated Mr. RODRIGUEZ-Espinoza, observed fixed and dilated pupils, ordered an emergent CT scan, confirming an SDH, posterior ischemia (inadequate blood supply to the brain), and loss of brainstem reflexes. The neurosurgeon admitted Mr. RODRIGUEZ-Espinoza into the ICU, placed him on life support (use of specialized equipment to support life after the failure of one or more organs), and advised against surgery, due to irreversible loss of brain function and the poor chance of survival.

On September 9, 2019, Mr. RODRIGUEZ-Espinoza's repeat CT scan revealed extensive irreversible brain damage due to SDH, herniation (brain tissue, blood and cerebrospinal fluid shifts from their normal position inside the skull), multiple strokes, untreatable cerebral edema (brain swelling), and a poor chance of a meaningful recovery. The neurologist documented Mr. RODRIGUEZ-Espinoza's GCS of 4 and considered him a high risk for neurological deterioration, leading to imminent brain death.

On September 10, 2019, the NMCDH's MD disconnected Mr. RODRIGUEZ-Espinoza from life support and pronounced him dead at 9:35 p.m.

The final autopsy report revealed Mr. RODRIGUEZ-Espinoza's cause of death was complications of chronic alcoholism. The coroner noted Mr. RODRIGUEZ-Espinoza suffered from chronic alcoholism, cirrhosis of the liver, and an acute chronic SDH. Although each of these entities may cause death on its own, cirrhosis may contribute to SDH, as it is associated with increased risk of bleeding due to a decreased production of clotting factor.

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Mortality Finding(s):

A mortality review committee (MRC) unanimously determined that Mr. RODRIGUEZ-Espinoza's medical care at MCACF was not provided within the safe limits of practice, and directly and indirectly contribute to his death. It was determined that a non-healthcare provider performed the initial assessment, but did not fully evaluate Mr. RODRIGUEZ-Espinoza's acknowledgment of drinking a six pack of beer daily, or refer him to a higher-level provider. Additionally, an LPN acknowledged awareness of Mr. RODRIGUEZ-Espinoza's alcohol consumption, but did not document a complete alcohol use assessment (i.e., drinking frequency, last alcoholic intake, history of alcohol withdrawal, CIWA score) or refer him to a higher-level provider.

Mortality Review Detailed Report:

On September 3, 2019, Homeland Security Investigations (HSI) encountered Mr. RODRIGUEZ-Espinoza in Chicago, IL, while conducting investigations for violations of the Immigration Nationality Act (INA). HSI arrested Mr. RODRIGUEZ-Espinoza for unknown legal status and transferred him to ERO Chicago at MCACF for overnight housing. On September 4, 2019, ERO transferred and processed Mr. RODRIGUEZ-Espinoza at the Broadview Service Staging Area (BSSA) in Broadview, IL. On the same date, ERO transferred Mr. RODRIGUEZ-Espinoza back to MCACF.

On September 11, 2019, ICE Health Service Corps (IHSC) received notification of the death of ICE detainee Roberto RODRIGUEZ-Espinoza. Mr. RODRIGUEZ-Espinoza, a 37-year-old Mexican male, was in ICE custody from September 3 to 10, 2019, and assigned to MCACF on the date of his death. Mr. RODRIGUEZ-Espinoza was pronounced dead at NMCDH in Winfield, IL, on September 10, 2019.

The IHSC Assistant Director requested a mortality review to learn from Mr. RODRIGUEZ-Espinoza's death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and, identify issues that require further study.

The following report is based on the findings of the MRC, which convened on December 19, 2019. The MRC consisted of the following members: Captain (CAPT) (b)(6),(b)(7)(C) MD, IHSC Eastern Regional Clinical Director, physician reviewer; CAPT (b)(6),(b)(7)(C) Nurse Practitioner-certified, IHSC Deputy Assistant Director of Health Care Compliance (DAD of HCC), administration and advanced practice provider (APP) reviewer; Commander (CDR) (b)(6),(b)(7)(C) registered nurse, IHSC Western Regional Nurse Consultant, nurse reviewer; and Lieutenant Commander (b)(6),(b)(7)(C), Physician Assistant-certified, APP reviewer.

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The review was based on the following information: 1) Mr. RODRIGUEZ-Espinoza's MCACF medical records and community hospital records; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. RODRIGUEZ-Espinoza's MCACF detention file; 5) Mr. RODRIGUEZ-Espinoza's death certificate and autopsy report; 6) audio-visual recordings from September 3 to 7, 2019; 7) an on-site review and staff interviews conducted by fact-finder CDR (b)(6),(b)(7)(C) at MCACF on November 13-15, 2019; and 8) applicable MCACF policies and procedures and ICE Detention Standards.

Although, MCACF is managed under a U.S. Marshall's rider, the detention standards used for this review were: ICE National Detention Standards (NDS) 2000, American Correctional Association (ACA) Standards, 2004 and the National Commission on Correctional Health Care (NCCHC) Standards, 2018. The following is a summary of health care delivery/program strengths, weaknesses, and recommendations found during this review:

Weaknesses

1. Intake Screening.

- On September 3, 2019, MCACF custody and medical did not complete a thorough evaluation of Mr. RODRIGUEZ-Espinoza's alcohol use.
- On September 3, 2019, VS, or a CIWA score, were not completed at intake to assess possible alcohol withdrawal.
- VS were not documented.

Applicable standards of care for this finding:

- NDS 2000: Medical Care; section D. *Medical Screening (New Arrival)*.
- ACA standard, 1-HC-1A-19, *Health Screens*.
- NCCHC standard, J-E-02, *Receiving Screening*.
- MCACF policy, HCD 100-E-02, *Receiving Screening*, May 2019.

2. Inadequate Referrals.

- On September 3, 2019, MCACF's medical staff did not refer Mr. RODRIGUEZ-Espinoza timely or appropriately to a higher level of care for further evaluation of his alcohol history.

Applicable standards of care for this finding:

- MCACF policy, HCD 100-E-02, *Receiving Screening*, May 2019.

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- MCACF policy, HCD 100-F-04, *Medically Supervised Withdrawal and Treatment*, May 2019.

3. Documentation.

- MCACF's medical staff did not appropriately and consistently document Mr. RODRIGUEZ-Espinoza's medical care while at MCACF (i.e., VS, CIWA score, and thorough intake assessment).
- Documentary evidence did not reflect when the LPN initially contacted the on-call physician regarding Mr. RODRIGUEZ-Espinoza's condition.
- The LPN documented a CIWA score of 4; however, it was unclear how the CIWA score was determined based on the parameters.

Applicable standards of care for this finding:

- MCACF policy, HCD 100-F-04, *Medically Supervised Withdrawal and Treatment*, May 2019.

4. Timely reading of Tuberculin Skin Test

- On September 3, 2019 at 11:00 p.m., an LPN administered Mr. RODRIGUEZ-Espinoza's tuberculin skin test, and the results were read on September 5, 2019 at 9:00 a.m., which is less than the required time frame of 48-72 hours to check for induration at the site.

Applicable standards of care for this finding:

- Centers for Disease Control (CDC) and Prevention: Morbidity and Mortality Weekly Report: *Prevention and Control of Tuberculosis in Correctional and Detention Facilities: Recommendations*, July 2006.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance (HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, the MCACF administrator, and health authority for review and any action(s) deemed appropriate.

End of report.